# Interview Guide: Barangay San Antonio Health Office

*TOGAF Preliminary Phase · UA&P Solution Architecture · Stakeholder Discovery*

**Interviewee name:** _____ **Position / role:** _____

**Date & time:** _____ **Interviewers:** _____

## Part 1: What to Say at the Start (about 3 minutes)

### What is Solution Architecture?

> "Before anyone builds a house, an architect draws the blueprint. The blueprint shows where the rooms go, where the pipes run, and how the doors connect. It also shows how the house will be used by the people living in it.
>
> **Solution Architecture is the blueprint, but for how an organization works.** We look at three things together:
>
> 1. **The people and their tasks.** Who does what: the midwife, the BHWs, the nutrition scholar.
> 2. **The information.** What gets written down: patient records, immunization lists, weight monitoring.
> 3. **The tools.** Where it gets written: logbooks, Excel, Google Forms, government systems.
>
> Then we ask: *where do these not fit together, and what would a better setup look like?*"

**A simple example:** "If a mother's baby is recorded in three different logbooks, and one BHW is the only person who knows where they all are, what happens when that BHW goes on leave? That kind of gap is what we're looking for."

### Why TOGAF?

> "TOGAF is like a **recipe book that professional architects around the world use**. It's a step-by-step method, so we don't skip anything important. It makes us:
>
> - **understand you first** before suggesting anything (that's today),
> - write down **the rules you care about**, like patient privacy, and
> - plan changes **in small, safe steps** instead of changing everything at once.
>
> We're using it because it's the standard taught in our class. It also keeps our recommendations based on *your* actual problems and not what we think is cool."

### What's the end goal?

> "At the end, we want to give you a **clear plan, a roadmap**. It will show:
>
> - how things work today,
> - what a better version looks like, and
> - which steps to take first, second, third.
>
> We are **not** selling you software, and we're not changing anything in your office. Think of it as a free check-up for how your health center runs. You can use the plan when asking the city for budget or support."

**Set expectations honestly.** Tell them it's a student project, the plan is theirs to use or ignore, and you won't collect any patient names or records. If they prefer Filipino, the blueprint analogy works too: *"Parang plano ng bahay bago itayo."*

## Part 2: Interview Questions

*Each group of questions feeds a specific part of the 2-page brief (shown in the green tag).*

### A. The Basics Q1.1 Enterprise Scope & Baseline

1. What is the official name of this office? Is it the Barangay Health Center, a Health Station, or something else?

2. How many people work here? Please name the roles, for example midwife, nurse, BHWs, Barangay Nutrition Scholar, and the Kagawad on Health. How many are full-time and how many are volunteers?

3. How many sites do you run? For example, the main center, satellite stations, or puroks you visit.

4. About how many residents or households do you serve? About how many patients visit in a normal week?

5. What services do you offer? For example, prenatal care, immunization, family planning, nutrition weighing (OPT), TB-DOTS, senior citizen checkups, and dengue response.

6. **Where do you write things down?** Please list everything: logbooks, Target Client Lists, Excel files, Google Forms, Facebook/Messenger groups, iClinicSys, FHSIS, PhilHealth Konsulta, or other DOH or city systems.

7. What equipment do you have? How many computers? Is the internet reliable? Is there a printer, and are there phones the office provides?

8. Are you using any software, laptop, or system that is old, breaking, or about to lose support or funding?

### B. The Main Problem Q1.2 Business Catalyst

9. What is the **biggest headache** in your work right now?

10. Are there deadlines or reports you have to submit to the City Health Office or DOH? How hard are they to complete on time?

11. Have you ever been warned or corrected, or lost funding, because of late or wrong reports?

12. Are there new rules you now have to follow? For example, the Data Privacy Act, the UHC Act, or PhilHealth Konsulta requirements.

13. Is anything about to change? For example, staff leaving, a new barangay administration, a budget cut, or more patients than before.

### C. When Things Go Wrong Q1.3 Failure Modes

14. Tell me about a **specific time** something went wrong. For example, a lost record, a patient whose history couldn't be found, a double-counted vaccine, or a report that had to be redone.

15. If the key person (e.g., the midwife) were absent for a month, what would stop working?

16. How long does it take to prepare your monthly report? Do you have to copy the same data into more than one place?

17. How do you find a patient's past records today, and how long does it take?

### D. Who's in Charge Q2.1 & Q2.2 Governance & RACI

18. Who is the head of this office? Who do you report to at the barangay, and who at the City Health Office?

19. Who has the final say on changes, like a new form, a new process, or a new system? *(This person is your Executive Sponsor / Accountable.)*

20. Who would actually do the work if something changed? *(This is your Responsible.)*

21. Who should be asked for their opinion, and who only needs to be informed? *(This is your Consulted / Informed.)*

22. Which outside offices do you depend on? For example, the City Health Office, DOH, PhilHealth, the DSWD, schools, or hospitals you refer patients to.

### E. Your Calendar Q2.3 Framework Interface

23. What are your busy seasons? For example, immunization drives, Garantisadong Pambata, Operation Timbang (around January to March), or dengue season.

24. When are reports due: monthly, quarterly, or yearly?

25. When does the barangay plan its budget? *(This tells you when your recommendations could actually get funded.)*

26. Do you follow a written manual or standard procedure? Is there a DOH or city quality standard you're measured on?

### F. What Matters Most to You Q4.1 Principles

27. If you could fix **one thing**, what would it be?

28. How do you keep patient information private today? Who is allowed to see the records?

29. What would you *never* want to change, even if a new system came in?

30. Would you choose a simpler system that works offline, or a more powerful one that needs internet?

### G. Proof Q4.2 Anti-Fabrication Evidence

31. Can we write down your name and position, and today's date, as our source?

32. Can you confirm a few numbers for us? For example, staff count, residents served, patients per month, and number of logbooks or systems.

33. Can we see a **blank** copy of the forms or logbooks you use? Please, no patient information.

## Tips for the Interview

- **Always ask for numbers.** "How many?" and "how long?" are what the grader's red-flag table checks for.
- **Write down the interviewee's role and the date.** The brief requires this.
- **Never take photos of real patient records.** Blank forms only. This also protects you under the Data Privacy Act.
- **Ask "Can you give me an example?"** after any general answer. Specific stories become your failure modes.